

North East London

Treatment Pathway for Inflammatory Bowel Disease
in adults

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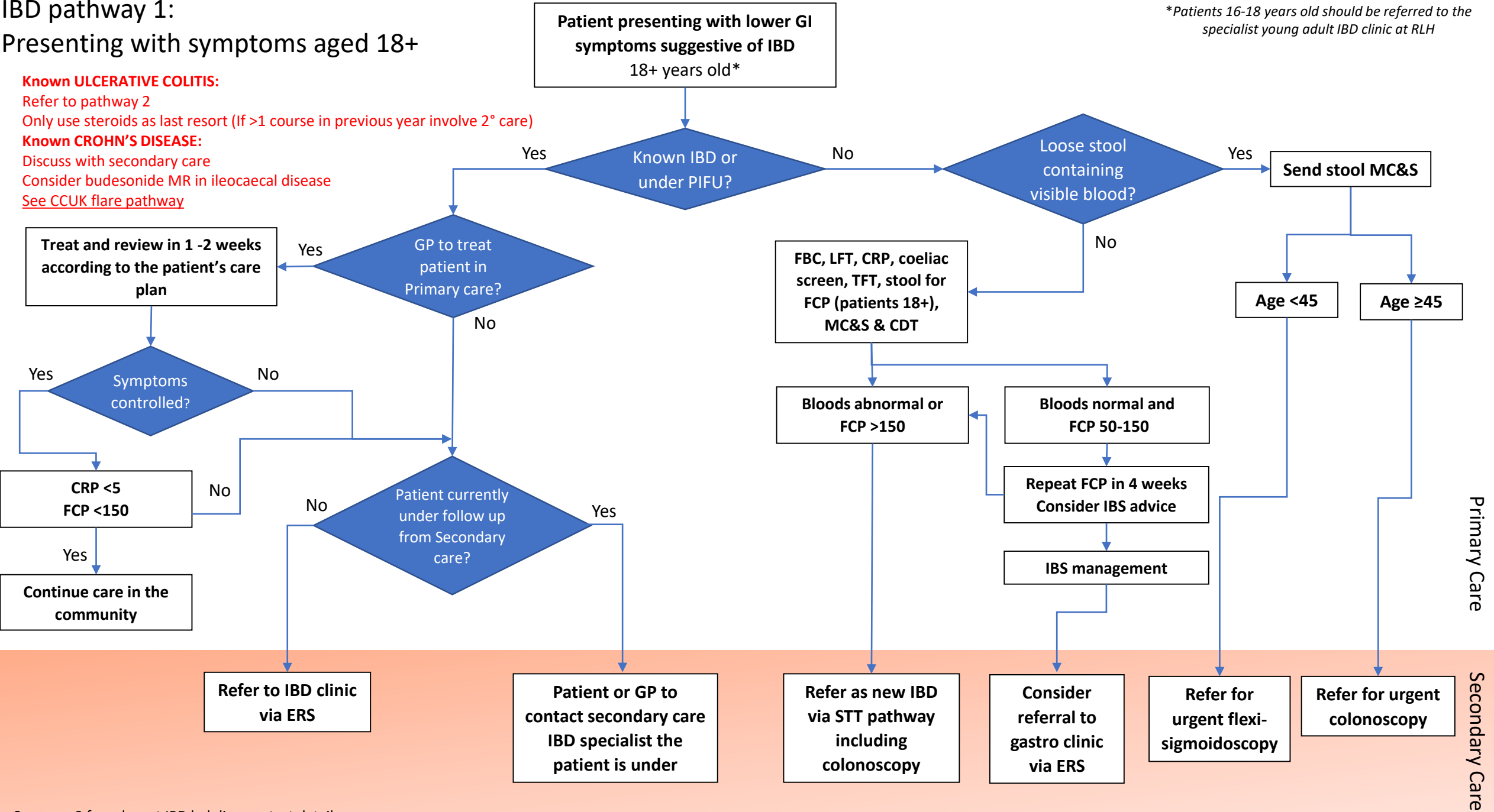
With thanks and acknowledgement to South East London ICS, their IBD pathway was adapted in the production of this pathway.
NB: This pathway is correct at the time of publication. Any NICE Technology Appraisals which are published after this date in relation to IBD (adults) will be commissioned in line with the TA implementation recommendations.

**Patients 16-18 years old should be referred to the specialist young adult IBD clinic at RLH*

IBD pathway 1: Presenting with symptoms aged 18+

Known ULCERATIVE COLITIS:
Refer to pathway 2
Only use steroids as last resort (If >1 course in previous year involve 2° care)

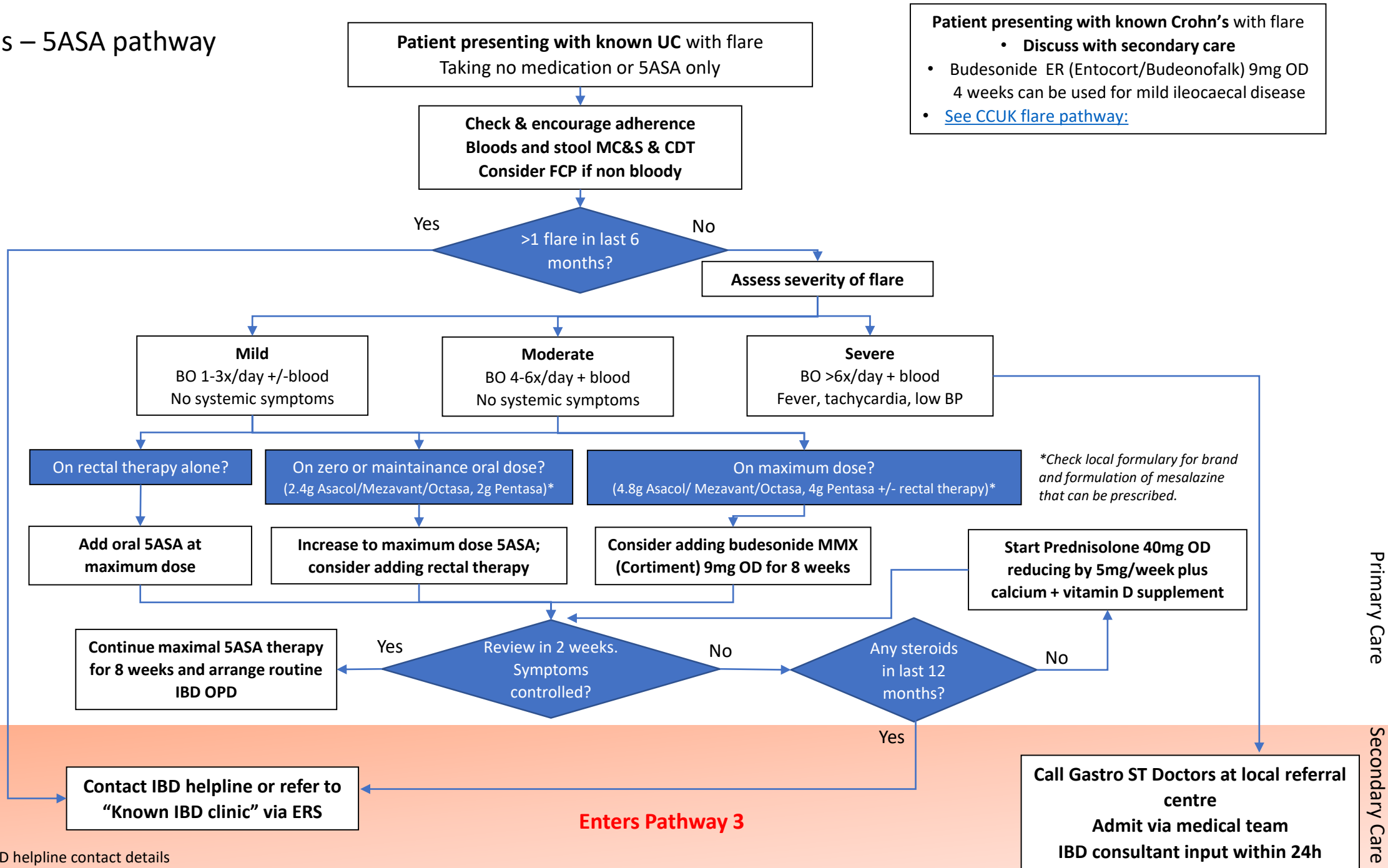
Known CROHN'S DISEASE:
Discuss with secondary care
Consider budesonide MR in ileocaecal disease
[See CCUK flare pathway](#)



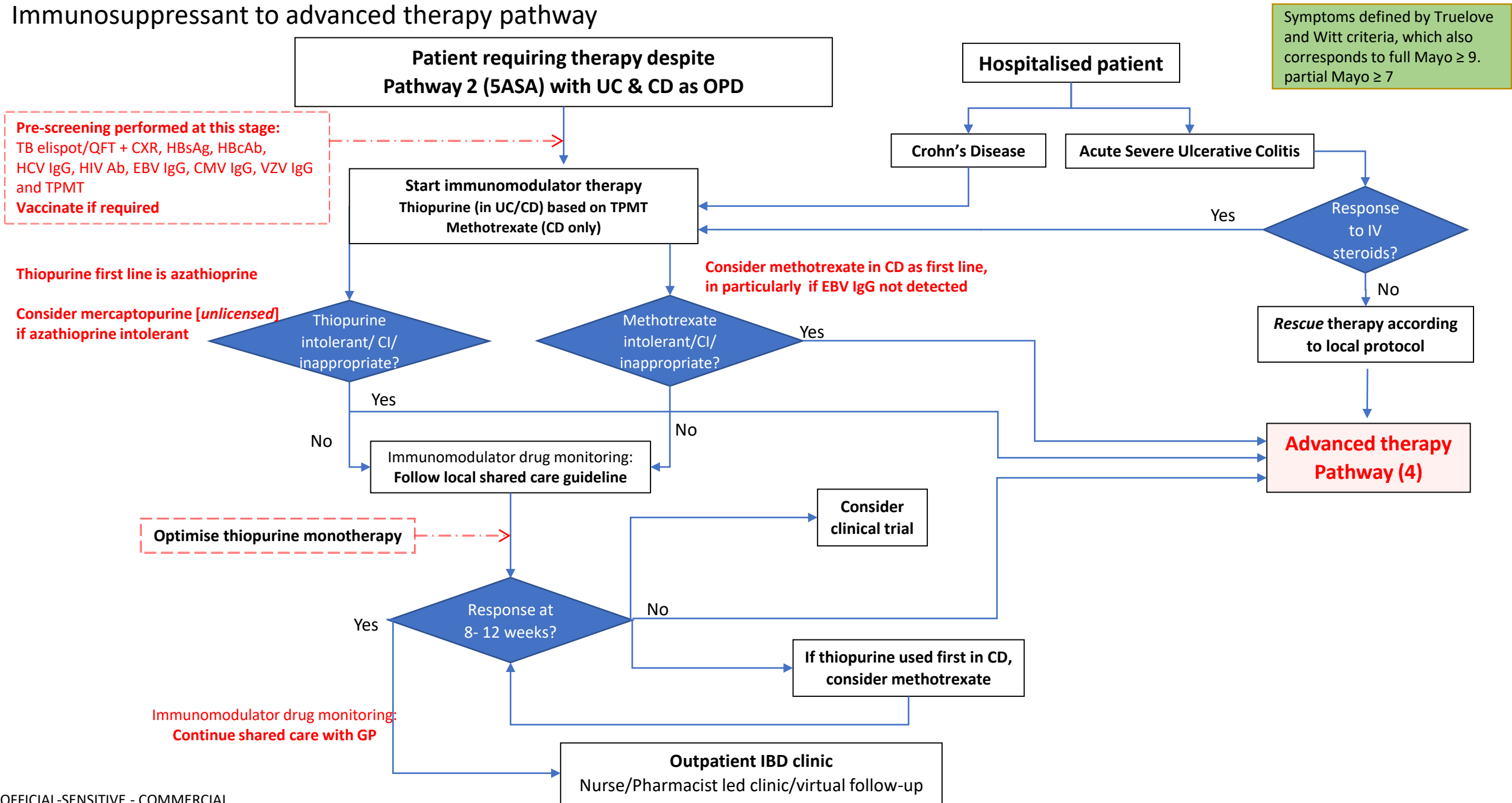
IBD pathway 2:
Ulcerative colitis – 5ASA pathway

Rectal therapy:
- Suppository for proctitis
- Enema for left sided UC

Enema *can* be added for pancolitis

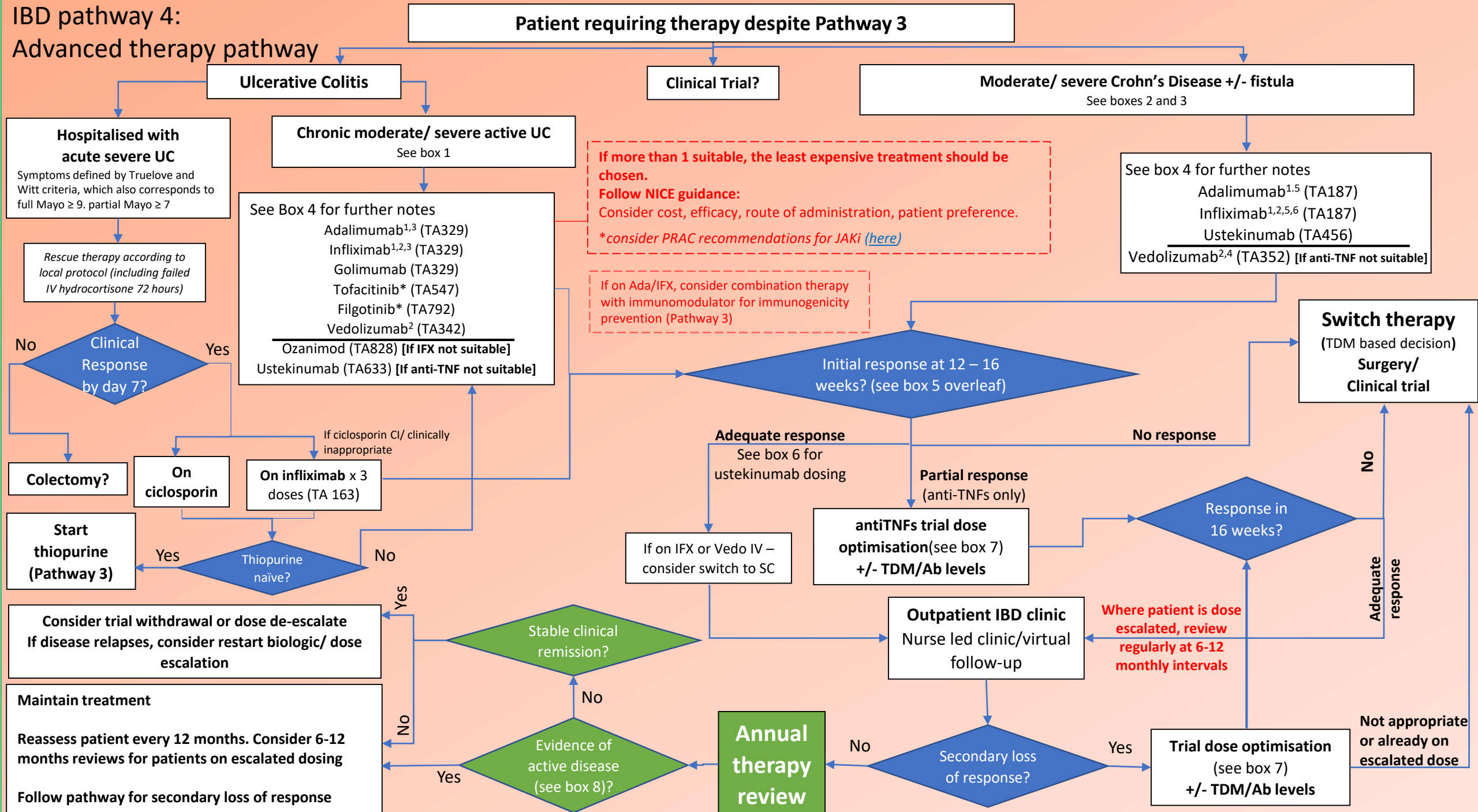


IBD pathway 3
Immunosuppressant to advanced therapy pathway



IBD pathway 4:

Advanced therapy pathway



IBD pathway 4: Advanced therapies pathway (notes)

Box 1. Ulcerative colitis- access criteria and definition of disease

Patients would need to have had:

- Inadequate response/ intolerance/ contraindication to optimised conventional therapy taken for an adequate period, including:
 - Corticosteroids **and/or**
 - Azathioprine/ 6-mercaptopurine
- Moderate to severe active UC, **normally corresponds to a Mayo score ≥ 6 , partial Mayo score ≥ 5 or SCCAI ≥ 6**

If an alternative disease severity scoring system is used, evidence of correlation with disease severity (e.g. endoscopy or radiology results, faecal calprotectin) and response criteria needs to be provided by the clinician.

Box 2. Crohn's Disease access criteria and definition of disease

Patients would need to have had:

- Inadequate response/ intolerance/ contraindication to optimised conventional therapy taken for an adequate period, including:
 - Immunosuppressants (e.g. azathioprine/6-mercaptopurine/methotrexate) **and/or**
 - Corticosteroids
- Moderate to severe active CD, **normally corresponds to a Crohn's disease activity index (CDAI) score ≥ 220 or Harvey-Bradshaw (HBI) score ≥ 6**

If an alternative disease severity scoring system is used, evidence of correlation with disease severity (e.g. colonoscopy, stoma output, faecal calprotectin) and response criteria needs to be provided by the clinician.

Box 3: Fistulising Crohn's disease- treatment options

Infliximab is the only treatment option (NICE TA 187), provided that the disease has not responded to conventional therapy (including antibiotics, drainage and immunosuppressive treatments), or who are intolerant of or have contraindications to conventional therapy. From clinical practice, it is very rare that patients would present with pure fistulising disease without meeting the moderate/ severe Crohn's criteria

Box 4: Choice of therapy

¹Biosimilars available. Use best value brand.

²Subcutaneous = clinically approved and commissioned locally. Not evaluated by NICE TAs

³Consider combination therapy with immunomodulator for immunogenicity prevention (Pathway 3)

⁴Only if anti-TNF failed/ not tolerated/ contraindicated as per NICE TAs

⁵Moderate disease not included in the economic analysis for NICE TA187 but clinically approved and commissioned locally (pending)

⁶Infliximab is first choice in perianal disease

Box 5: Definition of response

Adequate response (UC):

Complete Mayo:

- decrease in full Mayo score from baseline by ≥ 3 points and $\geq 30\%$, AND
- decrease in rectal bleeding sub-score from baseline by ≥ 1 point, OR absolute rectal bleeding sub-score of 0 or 1.

Partial Mayo (where further endoscopy not considered necessary/appropriate):

- decrease in partial Mayo score from baseline of ≥ 2 points and $\geq 25\%$ AND
- decrease in rectal bleeding sub-score from baseline of ≥ 1 point OR absolute rectal bleeding sub-score of 0 or 1.

Adequate response (CD) - decrease in HBI ≥ 3 points or CDAI ≥ 70 points

•**Partial response**- any improvement in HBI/CDAI/Mayo/partial Mayo that does not meet adequate response criteria

•**No response** – worsening/ no change of HBI/CDAI/ Mayo/partial Mayo

If alternative disease severity scoring system used, evidence of treatment response (e.g. endoscopy or radiology results, faecal calprotectin) to be provided.

Box 6: Ustekinumab dosing post-initiation:

Ustekinumab can be administered every 8 weeks or 12 weeks according to SPC post-initiation provided **adequate response is demonstrated. If patient is not in remission by week 14, use 8 weekly dosing.**

Box 7: Trial dose escalation, review for clinical response at 12 - 16 weeks

Note only infliximab and adalimumab dose escalation is allowed **if partial response during induction period**

Infliximab: 10mg/kg 8 weekly OR 5mg/kg 4 weekly OR 5mg/kg 6 weekly

Adalimumab 40mg weekly

Tofacitinib 10mg twice daily (consider VTE risk)

Ustekinumab 90mg 8 weekly

Vedolizumab 300mg 4 weekly. (Use of vedolizumab TDM for dose adjustments is unvalidated at time of writing)

Box 8: Disease reassessment at 12 months

Treatment should only be continued if there is evidence of ongoing adequate response and active disease, or it is considered clinical inappropriate to withdraw therapy. Ongoing active disease may be determined by:

- Clinical symptoms and
- Biological markers and
- Investigations, including endoscopy, imaging if necessary.

Clinical Remission:

UC: Normally defined as complete Mayo ≤ 2 with no subscore >1 , partial Mayo ≤ 1 or SCCAI ≤ 2

CD: Normally defined as HBI ≤ 4 or CDAI ≤ 150

Appendix 1. Drug factors to consider (including modes of action)

The table below provides an approximate drug cost for each biologic based upon first year of therapy, with the loading dose schedule taken into consideration. The cost will vary depending upon commercial arrangements and access to short-term free of charge supplies, which has not been taken into consideration for this guidance.

Mode of action	Drug name	Indicated for	TA (other indications)	Reviewed within	Drug cost
Anti-TNF α	Adalimumab	<u>TA187 – Crohn’s disease</u> Severe active Crohn’s: which has not responded/ intolerant/ contraindication to conventional therapy (immunosuppressive and/or corticosteroid treatments)	TA199 – Psoriatic arthritis TA195 – Rheumatoid arthritis TA373 – JIA TA375 – Rheumatoid arthritis TA383 – Ankylosing spondylitis and non-radiographic axial spondylitis TA392 – Hidradenitis suppurativa TA715 – Rheumatoid arthritis	12 weeks	£
		<u>TA329 – Ulcerative colitis</u> Moderate to severe active ulcerative colitis: which has not responded/ intolerant/ contraindication to conventional therapy including corticosteroids and mercaptopurine/ azathioprine.		2-8 weeks	
	Golimumab	<u>TA329 – Ulcerative colitis</u> Moderate to severe active ulcerative colitis: which has not responded/ intolerant/ contraindication to conventional therapy including corticosteroids and mercaptopurine/ azathioprine.	TA220 – Psoriatic arthritis TA225 – Rheumatoid arthritis TA375 – Rheumatoid arthritis TA383 – Ankylosing spondylitis and non-radiographic axial spondylitis TA497 – Ankylosing spondylitis	12-14 weeks	££
	Infliximab (biosimilar) SC or IV	<u>TA163 – Ulcerative colitis (acute)</u> Acute exacerbations of ulcerative colitis: which has not responded/ intolerant/ contraindication to conventional therapy including corticosteroids and mercaptopurine/ azathioprine. <u>TA329 – Ulcerative colitis</u> Moderate to severe active ulcerative colitis: which has not responded/ intolerant/ contraindication to conventional therapy including corticosteroids and mercaptopurine/ azathioprine.	TA195 – Rheumatoid arthritis TA199 – Psoriatic arthritis TA375 – Rheumatoid arthritis TA383 – Ankylosing spondylitis and non-radiographic axial spondylitis TA715 – Rheumatoid arthritis	3 doses (acute ulcerative colitis) 14 weeks (ulcerative colitis)	£
		<u>TA187 – Crohn’s disease</u> Severe active Crohn’s: which has not responded/ intolerant/ contraindication to conventional therapy (immunosuppressive and/or corticosteroid treatments) Active fistulising Crohn’s disease which has not responded/ intolerant/ contraindication to conventional therapy (including antibiotics, drainage and immunosuppressive treatments).		2 doses (Crohn’s disease) 3 doses (fistulising Crohn’s disease)	

Mode of action	Drug name	Indicated for	TA (other indications)	Reviewed within	Drug cost	
JAK inhibitors	Tofacitinib	<u>TA547 – Ulcerative colitis</u> Moderately to severely active ulcerative colitis when conventional therapy of biological agent cannot be tolerated, or the disease has responded inadequately or lost response to treatment.	TA735 – JIA TA480 – Rheumatoid arthritis TA543 – Psoriatic arthritis TA676 – Rheumatoid arthritis	16 weeks	££	
	Filgotinib	<u>TA792 – Ulcerative colitis</u> Moderately to severely active ulcerative colitis: which has not responded/ intolerant/ contraindication to conventional or biological treatment.		10 weeks	£	
IL-12 and IL-23 inhibitor	Ustekinumab	<u>TA456 – Crohn’s disease</u> Moderately to severely active Crohn’s disease when conventional therapy or a TNF-alpha inhibitor cannot be tolerated, or the disease has responded inadequately or lost response to treatment.	TA180 – Psoriasis TA340 – Psoriatic arthritis	16 weeks	£££	
		<u>TA633 – Ulcerative colitis</u> Moderately to severely active ulcerative colitis when a TNF-alpha inhibitor cannot be tolerated, or the disease has responded inadequately, lost response to treatment or is not suitable.				
IL-23 inhibitor	Vedolizumab SC or IV	<u>TA352 – Crohn’s disease</u> Moderately to severely active Crohn’s disease when a TNF-alpha inhibitor cannot be tolerated, or the disease has responded inadequately, lost response to treatment or is contraindicated. To be provided with the discount agreed in the patient access scheme.		14 weeks	££ SC	£££ IV
		<u>TA342 – Ulcerative colitis</u> Moderately to severely active ulcerative colitis when conventional therapy or a TNF-alpha inhibitor cannot be tolerated, or the disease has responded inadequately or lost response to treatment.To be provided with the discount agreed in the patient access scheme.		10 weeks		
Sphingosine 1-phosphate (S1P) receptor modulator	Ozanimod	<u>TA828 – Ulcerative colitis</u> Moderately to severely active ulcerative colitis when conventional treatment cannot be tolerated or is not working well enough and infliximab is not suitable, or biological treatment cannot be tolerated or is not working well enough and the company provides it according to the commercial arrangement.	TA706 – Relapsing-remitting multiple sclerosis	10 weeks	£££	

Pathway 1

Abbreviation	Name
CDT	Clostridium difficile test
GI	Gastrointestinal
IBD	Inflammatory bowel disease
MC&S	Microscopy, culture and sensitivity
FCP	Faecal calprotectin
FBC	Full blood count
LFT	Liver function test
CRP	C-reactive protein
TFT	Thyroid function test
IBS	Irritable bowel syndrome
ERS	Electronic referral system
STT	Straight to test
PIFU	Patient Initiated Follow-Up

Pathway 4

Abbreviation	Name
Ada	Adalimumab
IFX	Infliximab
Vedo	Vedolizumab
SC	Subcutaneous
TDM	Therapeutic drug monitoring
Ab	Antibody
IBD	Inflammatory bowel disease

Pathway 2

Abbreviation	Name
BO	Bowels open
BP	Blood pressure
5ASA	Aminosalicylates
OD	Once daily
OPD	Out-patient department

Pathway 3

Abbreviation	Name
HBsAg	Hepatitis B surface antigen
HBcAb	Hepatitis B core antibody
HCV IgG	Hepatitis C virus antibody
TB elispot/QFT	Tuberculosis elispot/quantiferon test
HIV Ab	Human immunodeficiency virus antibody
EBV IgG	Epstein-Barr virus antibody
CMV IgG	Cytomegalovirus antibody
VZV IgG	Varicella zoster virus antibody
TPMT	Thiopurine S-methyltransferase
IV	Intravenous
CI	Contraindicated
MTX	Methotrexate

IBD contact details

Barts Health NHS Trust

Royal London and Mile End hospitals:

Adult service Tel: 020 3594 3700 email: bhnt.ibdhelpline@nhs.net

Paediatric service Tel: 020 3594 0402 email: bartshealth.pibd.helpline@nhs.net

Whipps Cross hospital

Tel: 0208 539 5522 ext 4210 email: bartshealth.wxhibdhelpline@nhs.net

Newham hospital

Tel: 07761405192

Homerton Healthcare NHS Foundation Trust

Adult service Tel: 0208 5105906 email: Huh-tr.homertonibdcns@nhs.net

Barking , Havering and Redbridge University Trust:

Adult service Tel: 01708 435 000 ext 3090 or 3673 email: bhrut.ibdhelp@nhs.net